

Using the Best Value SGLT2 Inhibitors (-Flozins) Scheme January, v1.1

1. Scheme Outline

The purpose of this scheme is to support better value prescribing of Sodium-glucose Co-transporter-2 (SGLT2) inhibitors for patients with type 2 diabetes across Devon, in line with NICE guidance.

This scheme will run to 30th April 2026.

2. Background

2.1 National Outlook

SGLT2 inhibitors, sometimes referred to as “-flozins”, are increasingly prescribed for the management of Type 2 diabetes (T2DM). They are recommended by NICE both as monotherapy (where metformin is contraindicated or not tolerated) and in combination with other antidiabetic drugs¹.

Their role has expanded further with NICE guidance recommending use in chronic kidney disease (CKD) and heart failure (HF)²⁻⁴. Currently, several SGLT2 inhibitors are available to prescribe: canagliflozin, dapagliflozin, empagliflozin, and ertugliflozin. NICE recommends that if two drugs in the same class are appropriate, choose the option with the lowest acquisition cost¹.

In the 12 months to October 2025, NHS Devon spent £23.64m on antidiabetic drugs (excluding insulin), of which, £10.35m was spent on SGLT2 inhibitors (44%). Prescribing data shows that the average cost per SGLT2 item in Devon was 2% higher than national average.

2.2 Patent expiry

In 2025, dapagliflozin moved from Category C to Category M of the Drug Tariff and the acquisition cost reduced significantly in comparison to other available SGLT2 inhibitors (*Table 1*)⁵. The other commonly used SGLT2 inhibitors in Devon, such as canagliflozin, empagliflozin, and ertugliflozin, remain listed under category C.

Table 1 - Comparison of January 2026 drug tariff prices for available SGLT2 inhibitors

SGLT2 inhibitor	Drug tariff prices	
Dapagliflozin	£7.22 (28, 10mg tablets)	£7.58 (28, 5mg tablets)
Canagliflozin	£39.20 (30, 300mg tablets)	£39.20 (30, 100mg tablets)
Empagliflozin	£36.59 (28, 25mg tablets)	£36.59 (28, 10mg tablets)
Ertugliflozin	£29.40 (28, 15mg tablets)	£29.40 (28, 5mg tablets)

2.3 Dosing information

In undertaking this review and change activity, the practice acknowledges that it accepts clinical governance responsibility for completing this work.

The dose of dapagliflozin is dependent on the patient's indication and their clinical circumstances. The standard adult dose for dapagliflozin is 10mg once daily. Dose adjustments are not generally required in renal impairment: however, it is not recommended to initiate treatment with dapagliflozin in patients with eGFR less than 15 mL/min/1.73m².

Additionally, dose adjustment may be required in hepatic disease.

Practices should ensure that relevant blood results are current before amending prescribed medication. The onus is on the practice to decide if existing results can be used for each patient, considering the patients' individual circumstances, and the required frequency of their blood tests. In some instances, new blood tests may be required.

2.4 Inclusion and exclusion criteria:

The Devon formulary describes that generic dapagliflozin should be regarded as the first line choice SGLT2 inhibitor for all indications other than CKD in patients *without* type 2 diabetes, although the prescriber should consider the patient's clinical appropriateness, as per NHS England recommendations.

NHS England state that for the time being, treatment of CKD in patients *without* T2DM should be considered a patent protected indication, where the AstraZeneca brand Forxiga should be prescribed if dapagliflozin is required for the patient (see [link](#))

Patient cohorts	Generic dapagliflozin eligibility
T2DM (may have other co-morbidities including CKD and HF)	Include
HF	Include
CKD <i>without</i> T2DM	Exclude

3. Summary of Scheme

Practices are asked to review patients prescribed non-preferred SGLT2 inhibitors. It is the responsibility of the practice to determine how this may be best achieved from a clinical and operational governance perspective. Non-preferred SGLT2 inhibitors are: canagliflozin, empagliflozin, and ertugliflozin. Practices are asked to exclude patients who have CKD without T2DM.

Following the review of patients prescribed non-preferred SGLT2 inhibitors, if clinically appropriate, the practice is asked to make a prescribing change to generic dapagliflozin and communicate this change to the patient.

Practices will be remunerated where dapagliflozin is prescribed as a standard generic preparation listed in the Drug Tariff. Prescriptions for premium-priced or manufacturer-specific generics (commonly referred to as 'ghost generics') are not eligible for inclusion in this scheme.

3.1 Scheme Activity

The actions for practices are as follows:

Action 1 – Clinical System Searches

Import and run clinical system searches provided by the medicines optimisation team, to identify patients prescribed non-preferred SGLT2 inhibitors.

Action 2 – Patient Reviews

All patients identified in the clinical system searches should be reviewed, and where it is clinically appropriate prescribing should be changed from non-preferred SGLT2 inhibitors to generic dapagliflozin.

Action 3 – Data Submission

Complete the [Microsoft form](#) detailing the activity undertaken for patients identified as prescribed a non-preferred SGLT2 inhibitor within the practice. The completed form will need to be submitted for payment to be claimed, as detailed in section 5 (payment).

4. Eligibility

All practices within Devon are invited to engage with this scheme where there are patients prescribed non-preferred SGLT2 inhibitors.

Practices do not need to sign up for the scheme and remuneration will be based on submission of evidence and subsequent prescribing data as outlined in section 5 (Payment).

5. Payment

5.1 Standard payments

Practices will receive remuneration based on the number of reviews conducted and the changes to prescribing made:

- a. Practices will receive £10 per review for patients prescribed a non-preferred SGLT2 inhibitor.
- b. Practices will receive an additional £15 per patient where a change is made to generic dapagliflozin.

Where practices make a change from a non-preferred SGLT2 inhibitor to generic dapagliflozin they may claim payment a. and b. The maximum amount available for each patient identified is £25. There will be instances where a change is not possible for clinical reasons, in these cases a review payment only may be claimed.

Practices will need to complete clinical activity by 30th April 2026 to be eligible for remuneration as outlined above. Practices are required to submit the completed [Microsoft form](#), detailing the number of reviews and prescribing changes made for the standard payment by 31st May 2026.

The medicines optimisation team will confirm the standard payment amount and invite practices to invoice by 30th June 2026. Invoicing details are included in Appendix 1.

5.2 Additional payments

The additional payment provides the option to be remunerated at an extra £15 per patient, in addition to the standard payment of £25 per patient previously described. The additional payment is to incentivise rapid change of as many clinically appropriate patients as possible, during February and March 2026.

The additional payment will be based on the percentage of patients previously prescribed a non-preferred SGLT2 inhibitor, who are changed to generic dapagliflozin as set out in section 5.1. These figures will be measured by practice, using NHSBSA data for dispensed items during April 2026 and May 2026.

Table 2 - Additional payment by practice

Percentage of eligible patients (previously prescribed a non-preferred SGLT2 inhibitor) who are now prescribed generic dapagliflozin by practice	Remuneration per patient (previously prescribed a non-preferred SGLT2 inhibitor) who are now prescribed generic dapagliflozin
60% minimum achievement	£5
70% minimum achievement	£10
80% minimum achievement	£15

Therefore, for practices to avail themselves of these additional payments, changes will need to have been made rapidly so that patients may have their generic dapagliflozin items dispensed from 1st April 2026 onwards, at the latest.

Remuneration will be made to individual practices.

The additional payments will be calculated, and practices contacted by the medicine's optimisation team regarding any invoicing requirements during July 2026. Invoicing details are included in Appendix 1.

6. Further information

A summary of the key actions and timelines for practices are outlined in Appendix 2.

In the event of any disagreement, which cannot be resolved between the practice and the Medicines Optimisation team, the practice may forward an appeal describing the exceptional circumstances as to why achievement could not be attained within the specified dates described.

The Deputy Chief Pharmacist - Primary Care will constitute a panel consisting of NHS Devon and LMC representation, in equal numbers, to agree a resolution. Any appeal should be made to the Medicines Optimisation team by 31st August 2026.

7. References

1. National Institute for Health and Care Excellence. *Type 2 diabetes in adults: management (NG28)*. Available from: <https://www.nice.org.uk/guidance/ng28>
2. National Institute for Health and Care Excellence. *Dapagliflozin for treating chronic kidney disease (TA1075)*. Available from: <https://www.nice.org.uk/guidance/ta1075>
3. National Institute for Health and Care Excellence. *Dapagliflozin for treating chronic heart failure with preserved or mildly reduced ejection fraction (TA902)*. Available from: <https://www.nice.org.uk/guidance/ta902>
4. National Institute for Health and Care Excellence. *Dapagliflozin for treating chronic heart failure with reduced ejection fraction (TA679)*. Available from: <https://www.nice.org.uk/guidance/ta679>
5. NHS Business Services Authority. Drug Tariff – Pharmacies, GP practices and appliance contractors. NHSBSA. Available from: <https://www.nhsbsa.nhs.uk/pharmacies-gp-practices-and-appliance-contractors/drug-tariff>

Appendix 1 – Invoicing Information

The invoice needs to be on the surgery's headed paper.

- Please use a unique invoice number for each invoice submitted as it could be rejected if a similar number is used.
- Please state the description as 'Payment for Devon Medicines Optimisation SGLT2 Scheme 2026/27' detailing the number of patients reviewed, and the numbers of patients changed for the standard payment.
- Separate invoices should be submitted for standard payments (reviews/changes made) and for additional payments.
- For the additional payment component, further information will be supplied via email and the Medicines Optimisation reporting portal during July 2026.

Please include the following references:

- Cost centre: 958172-52114004
- Mark the invoice: XXLBAILEY to ensure correct assignment for processing.

Invoices may be submitted on Tradeshift or by mail to:

NHS Devon ICB
QJK Payables N055
PO Box 312
Leeds
LS11 1HP

Alternatively, invoices can be submitted as a non-editable pdf via email to sbs.apinvoicing@nhs.net

Please be advised, do not include the following wording within the subject line of your email: 'statement' or 'reminder'.

Appendix 2 – Summary of key actions and timelines for practices

Key action	Timeline
Import and run clinical system search to identify patients prescribed non-preferred SGLT2 inhibitors.	Before clinical activity commences
Review identified patients, and where clinically appropriate change prescribing to generic dapagliflozin as described in Section 3.1.	1 st February - 30 th April 2026 (Note: additional payment is based on dispensing data for February– March 2026)
Submit Microsoft form detailing activity undertaken within the scheme.	By 31 st May 2026
After confirmation of the standard payment amount from the medicines optimisation team, submit an invoice as detailed in section 5.1.	By 30 th June 2026
After confirmation of any additional payment amount from the medicines optimisation team, submit an invoice as detailed in Section 5.2.	By 31 st July 2026